

Question:

A baby is born after the 32nd gestational week by cesarean delivery. The mother suffered from gestational diabetes; however, she had no other pregnancy-related diseases and was otherwise healthy. The baby has a blood pressure of 100/58 mm Hg, heart rate of 104/min, and oxygen saturation of 88%. The child has tachypnea, subcostal and intercostal retractions, nasal flaring, and cyanosis. The cyanosis is responding well to initial administration of oxygen. The nasogastric tube was positioned without problems. Which of the following is the most likely diagnosis?

Answer Choices:

- A. Neonatal respiratory distress syndrome (NRDS)
- B. Tracheoesophageal fistula
- C. Pneumonia
- D. Congenital heart anomaly with right-to-left shunt

Reasoning + Answer:

<think> The patient is a newborn born at 32 weeks gestational age with no other pregnancy-related complications. The child has tachypnea, subcostal and intercostal retractions, nasal flaring, and cyanosis, which are indicative of respiratory distress. The cyanosis is responsive to oxygen, which is more consistent with an issue with oxygenation rather than a structural defect or infection. Neonatal Respiratory Distress Syndrome (NRDS) is the most common cause of respiratory distress in preterm infants, especially those born at 32 weeks gestational age. This condition is caused by a lack of surfactant, which helps to keep the alveoli open, leading to the symptoms described. Tracheoesophageal fistula would cause difficulty in feeding and positioning of the nasogastric tube, pneumonia would cause more localized signs of infection, and congenital heart anomalies with right-to-left shunt would typically cause persistent cyanosis that would not improve with oxygen.

</think>

<answer> A. Neonatal respiratory distress syndrome (NRDS) </answer>

